

[PROVIDER-NAME]
Consultant Gastroenterologist
[PROVIDER-NAME]hil MRCP
Consultant Gastroenterologist
[PROVIDER-NAME]
Consultant Gastroenterologist
Our Ref: (T) TS/cd/ [MED-ID]
[DATE]
Nutrition Clinic Letter [DATE]
[PROVIDER-NAME]
[ADDRESS]
[ADDRESS]
Totton, Southampton
Hampshire
[POSTCODE]
Division B
Care Group - Emergency & Specialist Medicine Gastroenterology
F Level, Room CF88, Mailpoint [ADDRESS]
[ADDRESS]
[POSTCODE]
Telephone: [PHONE]
Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME] , [ADDRESS]. [POSTCODE] DOB: [DATE] , Hospital Number: [MED-ID] ,
[NHS-NO]

Diagnoses:

Ulcerative colitis leading to colectomy and pouch formation

Pouch reconstruction

Pouchitis with cuffitis at EUA November 2009

Tendency to develop salt and water depletion

DVT's on long-term Warfarin

Epilepsy

I reviewed MISS [NAME] in the Intestinal Failure Clinic today. It is some years since I have
I thought she looked very well today. She is maintained on Ciprofloxacin 500 milligrams twice
has been effective with controlling her pouchitis although she still opens her bowels up to 1
day. When she was last seen by [NAME] she had a course of 5-ASA suppositories to treat her cu
unfortunately this resulted in problems with incontinence and a lot of discharge from her ana
I have suggested that an alternative antibiotic in the future if and when she gets a flare of
her pouchitis would be Augmentin. This may help limit her Metronidazole use. The other option
a therapeutic trial of probiotics in the form of VSL#3. In view of her current symptoms, I th
be helpful for her to have this at a dose of two sachets twice a day and I would be grateful
prescribe this for her for a one month trial. I hope this will then enable her to reduce the
Ciprofloxacin. I have warned her that the experience of using probiotics is variable and if t
evidence of any benefit after a four week trial then this should be stopped.

She also mentioned that she gets intermittent problems with abdominal cramping and finds Busc
helpful. She has been buying this over-the-counter and I would be grateful if you would add i
prescription. She also has frequent problems with nausea which may in part be related to her
therapy. She has had short courses of antiemetics in the past and again I would be grateful i
consider prescribing her a supply of Domperidone 10 milligrams to use on a PRN basis.

I have given her a form to repeat her bloods and urinary sodium prior to her next review in f
time.

With best wishes

Yours sincerely

[PROVIDER-NAME]

Consultant Gastroenterologist